



[Commonwealth of Massachusetts](#)
[Executive Office of Health and Human Services](#)
www.mass.gov/masshealth

December 2023 Update on MassHealth

Redetermination

Background on MassHealth redeterminations

[In March 2020, the federal government declared a public health emergency \(PHE\) due to the COVID-19 pandemic. In response to the PHE and consistent with federal requirements, MassHealth put protections in place that prevented members' MassHealth coverage from ending during the COVID-19 emergency. On April 1, 2023, these continuous coverage protections ended and MassHealth began the year-long process to redetermine all members' eligibility, as required by CMS.](#)

About the redeterminations data dashboard

[MassHealth is committed to publishing a monthly dashboard that provides data on MassHealth's caseload and detail on members joining and departing from MassHealth. This transparency will be critical to identifying populations that require additional support during the redetermination period and will give the public greater visibility into this important initiative. The dashboard will provide data through the end of the month prior to each release; for example, the December dashboard update contains enrollment data through the end of November 2023.](#)

Highlights from December 2023 dashboard

November Data

[Overall, during November 2023, the MassHealth caseload decreased by approximately 101,000 members, or 4.56%, from the prior month. Approximately 16,000 members newly gained coverage, approximately 12,000 re-joined coverage, and approximately 129,000 departed.¹](#)

- [• The ~16,000 arrivals are slightly lower than in past months.](#)
- [• The ~12,000 member re-openings reflect members who departed and re-joined MassHealth within 12 months. This represents a small fraction of all disenrolled individuals and is a far lower rate of re-opening than MassHealth observed before the COVID-19 Public Health Emergency.](#)
- [• The ~129,000 departures represent an expected increase in volume of departures as the redeterminations process continued for a seventh month. More information about this expected increase in closures through the end of the year can be found below \(See: Expected Increase in Closures through End-of-Year\).](#)
 - [o For context, prior to the COVID-19 Public Health Emergency, approximately 52,000 members departed MassHealth coverage each month \(based on data from CY2018 and CY2019\).](#)
 - [o Since April 2023, MassHealth has averaged approximately 57,000 departures per month.](#)
- [* MassHealth initiated redeterminations for approximately 187,000 members in November and is on track to initiate all 2.4M renewals during the 12-month "unwinding" period.](#)

¹ [Some members who join or re-join MassHealth are eligible for retroactive enrollment. As a result, caseload data reported in prior months' dashboard may fluctuate in each month's dashboard. For example, in last month's dashboard, we reported an October caseload of 2,289.4K members. This month's dashboard now shows 2,304.1K members in October, reflecting retroactive enrollments and reopenings.](#)

Aggregate Data since Beginning of Redeterminations in April

- Since redeterminations began in April, MassHealth has seen a net decrease of 8.43%, or ~203K members, in its caseload. MassHealth expects to see a substantial increase in departures in the months ahead, as more members go through the renewal process.
- Children have seen the lowest rate of disenrollment of any age group; enrollment of members under age 20 has been flat from April through the end of November, whereas adults' enrollment has declined.
- To date, ~38% of disenrolled members lost coverage because MassHealth confirmed that they are ineligible. ~59% of disenrolled members lost coverage due to insufficient information.
- While MassHealth's overall caseload has decreased by approximately 203K individuals since April 2023, the Massachusetts Health Connector's enrollment has grown by 48K. This suggests that a large portion of the individuals who have lost MassHealth eligibility are able to obtain affordable coverage through the MA Health Connector. This rate remains much higher than what is observed in other states.

Expected Increase in Closures through End-of-Year

MassHealth saw an expected increase in the number of departures for the month of November and anticipates December 2023 to see a significant number of departures as well, based in part on eligibility changes leading up to Massachusetts Health Connector's open enrollment process. Those eligibility changes are described in detail below:

- In August and September, MassHealth initiated renewals for members in mixed households (i.e., households where some members are on MassHealth, and some are on the Health Connector). Some members who are determined ineligible for MassHealth coverage during the mixed household renewal process, but are eligible for certain types of Health Connector coverage, receive additional time before disenrolling from MassHealth; they maintain MassHealth coverage for a longer period of time to better align with the Health Connector Open Enrollment Period, which ends in January 2024. This eases these members' transitions and minimizes any gaps in coverage. As a result, MassHealth expects December to see a significant amount of member departures.

How is MassHealth working to reach and support members?

Outreach efforts to members and collaboration with assisters (individuals available in the community to help members complete MassHealth renewals) have continued at a rapid pace.

- Through EOHHS's partnership with Health Care For All, canvassers have knocked on over 428K doors and community-based organizations have held over 2,800 events and activities in the 15 communities with the most members at risk of coverage loss.
- MassHealth Accountable Care Organizations and other health plans have made more than 1.1M outreach attempts via phone call, text message, and letter to members selected for renewal since April 2023. Additionally, MassHealth is partnering with health plans to directly assist members with completing renewal forms and applications.
- MassHealth is conducting a media campaign, which includes digital, traditional, and out of home ad placements, in over 30 communities with high MassHealth enrollment in the top four languages spoken by MassHealth members.
- Additionally, MassHealth has continued to expand member outreach efforts, including new member awareness efforts at 70+ grocery stores, ~600 libraries, ~1,800 schools, and additional statewide organizations such as the YMCA, Boys & Girls Club, etc.
- MassHealth has hosted several in-person renewal events in partnership with community organizations to support specific member populations through renewals, such as members who are experiencing homelessness.
- EOHHS held trainings specifically for eligibility specialists supporting members over age 65.
 - EOHHS executed >\$1.5M of grants to expand community assister capacity at 23 Community Based Organizations serving immigrants, refugees, older adults, and other vulnerable populations, as well as expanding assister capacity at Aging Services Access Points (ASAPs).

- EOHHS executed an additional \$1M grant in collaboration with the Health Connector to hire a team of 'Mobile Community Specialists' to conduct renewal assistance in the community.

You can learn more about MassHealth's renewal process at mass.gov/masshealthrenew Article information:

DeSantis's Oversight Board Said Disney Stripped It of Its Power—The New York Times
<https://www.nytimes.com/2023/03/30/business/disney-world-ron-desantis.html>

Article summary:

1. Florida Governor Ron DeSantis appointed a handpicked oversight board to restrict the autonomy of Disney World, but the board belatedly discovered that Disney had quietly sidestepped its control by pushing through a development agreement in early February.
2. The development agreement limits the power of the governor's appointees for decades to come and effectively circumvents their authority to govern, according to Brian Aungst Jr., a member of the council.
3. The governor's appointees have hired four law firms to investigate Disney's past behavior and potentially take legal action against the company, while Disney maintains that all agreements signed between them and the district were appropriate and approved in compliance with Florida's Government in the Sunshine law.

Article rating:

Appears moderately imbalanced: The article provides some useful information, but is missing several important points or pieces of evidence that would be required to present the discussed topics in a balanced and reliable way. You are encouraged to seek a more balanced perspective on the presented issues by exploring the provided research topics and looking at different information sources.

Article analysis:

The New York Times article titled "DeSantis's Oversight Board Said Disney Stripped It of Its Power" reports on the ongoing conflict between Florida Governor Ron DeSantis and Disney World over the company's self-governing privileges. The article highlights how Disney had quietly sidestepped the governor's efforts to restrict its autonomy by pushing through a development agreement that limits the power of the new oversight board appointed by DeSantis.

The article provides a detailed account of the events leading up to this conflict, including how DeSantis asked Florida lawmakers to terminate Disney World's special tax district privileges, which allow the company to self-govern its 25,000-acre theme park complex. The move was seen as retaliation for Disney's opposition to a new state education law that prohibits discussion about sexual orientation and gender identity in Florida classrooms.

However, the article also notes that the abolishment of the district would have required taxpayers in Orange and Osceola Counties to pick up the tab for some Disney World services, and so lawmakers passed a new measure allowing Disney to keep its special tax district while no longer being able to appoint members of its board. This is where Disney pushed through a development agreement limiting the power of DeSantis's new oversight board.

While the article provides a comprehensive overview of these events, it does not explore potential counterarguments or provide evidence for some claims made. For example, it suggests that DeSantis's appointees may have wanted to influence Disney's content offerings by using their power over development at the resort. However, there is no evidence presented to support this claim.

Additionally, some statements made in the article could be seen as biased or one-sided. For example, it notes that one board member has spread a baseless theory about tap water turning people gay and another is associated with a group that backed Florida's law restricting discussion of sexuality and gender identity in classrooms. These statements could be seen as attempts to discredit the board members and their views.

Overall, while the article provides a detailed account of the conflict between DeSantis and Disney, it could benefit from exploring potential counterarguments and providing evidence for some claims made. Additionally, some statements in the article could be seen as biased or one-sided.

Topics for further research:

- Counterarguments against DeSantis's efforts to restrict Disney's autonomy
- Evidence supporting claims of DeSantis's appointees wanting to influence Disney's content offerings
- Potential impacts of abolishing Disney's special tax district on taxpayers in Orange and Osceola Counties
- Disney's rationale for pushing through the development agreement limiting the power of DeSantis's oversight board
- Criticisms of the new state education law prohibiting discussion about sexual orientation and gender identity in Florida classrooms
- Implications of Disney's self-governing privileges on the local community and economy.

Report location:

<https://www.fullpicture.app/item/e039699a3d9590ac7d6bbd8f4aa56cfd>